



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : نعمه اسماعيل احمد اسماعيل
المحافظة : الجيزة
الحى / المدينة : امبابة
رقم البطاقة : 27402282102342 رقم التليفون : 01005119818
العنوان : ممم
العمر (تاريخ الميلاد) : 28-02-1974 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 29-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (.....)
المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط ثاني مثبتات
الاروماتيز [14161]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /د. /د. /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: نعمه اسماعيل احمد اسماعيل
National ID: 27402282102342



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor

الوقت :

--:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

نعمه
اسماعيل احمد اسماعيل
١٧ ش النضال - المعز لدين الله - المنيرة
اسكندرية - الجيزة

١٩٧٤/٠١/٢٨ ٢٧٤٠٢٢٨٢١٠٢٣٤٢

KJ2121744

٢٠٢٤/٠١/٢٨ ٢٧٤٠٢٢٨٢١٠٢٣٤٢

حاصلة على دبلوم تجارة
انثى
سليمان يوسف سليمان محمد
البطاقة سارية حتى ٢٠٢٤/٠١/٢٨

١



إدارة رئيس الجمهورية
لصحة المرأة المصرية

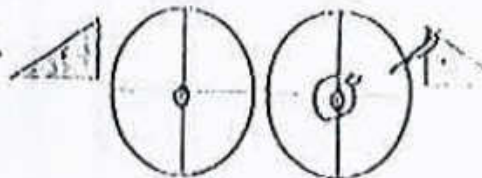
MDT FORM
J119

2.2.1
c.c.

0.01/10/2 MDT FORM

LF

Name	Dr. Mohamed El-Sayed
Age	47 years
MRN	
ID	502.5001.048
Sex	Male
Morbidity	1-0-1-1-1-1-1



Clinical Data

DX:	LDL	SM:	
UIC:		Mets. Workup:	
ER:	7/8		Free
PR:	5/8		
Her1:	1/8		
Ki67:	10%		
PT ² N ⁰ M ⁰			
COMMENT:			
LT WLE + LN (18/11/2021)			
+ LN PNI			

MDT OUTCOME & PLAN

For Adjuvant PORT
+ LF + Zoladex.

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
Dr. M. El-Sayed	Dr. El-Sayed	Dr. El-Sayed	

Patient Name : السيدة / أم محمد / أم محمد
Patient No. : 27402282102342
Path No. : 12833/21
Sex : Female
Age : 47 Y
Referring Doc :

Request No. : 137318
Request Date : 18/11/2021
Verification Date : 23/11/2021
Financial Category : التأمين - تأمين
Department : Out Patient

Pathology Report

Clinical Data:

Left breast cancer. Didn't receive chemotherapy.

Nature of the specimen:

Left WLE & axillary clearance.

Gross Examination:

Two specimens were received:

- 1- Left wire guided WLE specimen measured 9x5.5x4.5cm, partially covered by skin measured 6.5x3.5cm. Cut section revealed a firm greyish white mass measured 2.5x1.7x0.4cm, with least margin is 1cm (radial margin). Also there were very small multiple satellite subcutaneous nodules (5 nodules).
- 2- Axillary clearance specimens: A fibrofatty tissue piece measured 6.5x4x1cm, containing 17 lymph nodes.

Microscopic examination:

- Histological type: Remnants of invasive duct carcinoma grade II, including all satellite nodules (positive for invasion), and DCIS components of about 10%.
- Positive perinodal invasion
- Angiolymphatic: No
- Lymph node involvement: All lymph nodes are free (0/17).
- Resection margins: All margins are free

Diagnosis:

Left WLE & axillary clearance:
Remnants of invasive duct carcinoma grade II, with DCIS components of about 10%.
Positive invasion in satellite nodules.
Positive perinodal invasion.
All lymph nodes are free (0/17).
All margins are free
Stage: pT2, with multiple satellite nodules

Printing Date : 27/11/2021 12:14:08

Page 1 of 2

Printing By : marwa mohamed youss

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED:**

2. Chest

- Left breast, lateral parenchymal irregular mass lesion is seen measuring 2 x 1.3 cm.
- Right lower lung lobe (superior segment) pleural based nodular density is seen measuring 8 mm with other few tiny pulmonary nodules are also seen (2-3 mm).
- Lingular tiny benign calcific pulmonary nodule is also seen.
- Otherwise, No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis

- The liver is enlarged with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- Tiny gall bladder stone. No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals, apart from left adrenal tiny calcific focus.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Enlarged uterus with lobulated outline and myometrial enhancing mass measuring about 5 cm, likely fibroid.
- No abdominal or pelvic lymphadenopathy or collections.
- No suspicious lytic or sclerotic bone lesions.

OPINION:

- Left breast, lateral parenchymal irregular mass lesion.
- Few bilateral pulmonary micro-nodules likely of benign nature for follow up.
- Hepatomegaly. No focal lesions.
- Calculus gall bladder.
- Enlarged fibroid uterus.

Dr. Peter Nashat, MSc

Dr. Ayman Barouk, MD


 جمهورية مصر العربية
 بطاقة تحقيق الشخصية
 لعمه
 اسماعيل احمد اسماعيل
 ١٧ من النضال المعززين الله العظيمة
 امينية - الجيزة
 ١٩٧٤/٠٢/٢٨ ٢١ ٠٢٣ ٤٢
 KJ2121744

٢٠٢٤/٠٢/٢٨ ٢١ ٠٢٣ ٤٢
 حاصلة على دبلوم تجارة
 أنثى
 مسلمة
 سليمان يوسف سليمان
 البطاقة مصرية حتى ٢٠٢٤/٠٢/٢٨



جمهورية مصر العربية
بطاقة تحقيق الشخصية

نعمه
اسماعيل احمد اسماعيل
١٧ ش الفضل المعز لدين الله - المنيرة
امبابه - الجيزه

١٩٧٤/٠٨
٢٧٤٠٢٢٨٢١٠٢٣٤٢

KJ2121744

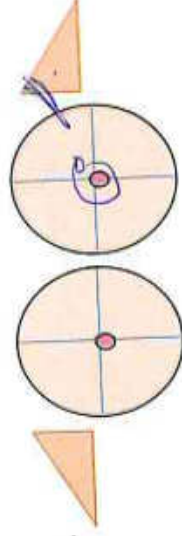




w/c c.c. 18/11/2021

MDT FORM

Name	د. هاشم
Age	47 years
MRN	27200000000000000000
ID	10000000000000000000
Co morbidity	



Clinical Data

Bx:	IDC.	SM:
IHC:		Mets Workup: Pre.
ER:	718	
PR:	518	
Her2:	Low	
KITV:		
	PT 2 N M	
COMMENT:	LT WLE + ALN C18/11/2021 + w PNI	

MDT OUTCOME & PLAN

For Adjuvant PORT
+ LF4 Zoladex.

SIGNATURE			PATHOLOGY
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	

C. c. 18/11/2021

Patient name:

MRN:

Diagnosis:

Unit:

Diet Type:

Diet Type:

[illegible]

any amendments, modifications, discontinuations, and corrections should be dated, timed, and signed.

* Do NOT use abbreviations.

***This form is NOT intended for use for medications orders.**

Patient name:

MIRN:

DOCTOR'S ORDERS

Diagnosis: BC

Unit:
Diet Type:Unit:
Diet Type:[illegible]

* All orders, modifications, discontinuations, and corrections should be dated, timed, and signed.

517.



Preval. left Breast

Lr BCS 18/11/2024

T2, N20/18, M2

Gn+7/6 pn+5/8, 1st+2+3, 1K167 15/

إدارة الجوده

وزارة الصحة والسكان
مركز أورام دار السلام
(هرمل سابقا)

Doctor's Orders

Rec. panel

or AE + 14/25/24

اسم المريض :

الرقم الطبي :

الأوامر

chest:.....

Temp:.....

Oral mucositis: 24/10/2024

Canula site infection.....

SH on 9/10/2024:

BENAS 2

o/e NAD

Rf femora 2.5 14/10/2024

التاريخ:

الوقت:

د/بناجع

2.5/ 14/10/24

توقيع الطبيب :

الأوامر

chest:.....

Temp:.....

Oral mucositis:.....

Canula site infection.....

Rf femora 2.5 14/10/2024

Call Alpha 1X1X30

Call Alpha 1X1X30

Gobalox D.S.S.C

Mlga 1X1X30

التاريخ: 14/10/24

الوقت:

توقيع الطبيب : د. / بناجع

Patient Name: Namh Ismail Ahmed Ismail	Date : 09/10/2025
Exam : MAMMOGRAPHY BOTH	Age : 51 Year(s)
Medical ID: 4480/2021	Pat. ID: 17618

Bilateral Digital soft tissue mammography with high resolution

breast sonography revealed:

Clinical summary: Post left CBS and ALAND followed by radiotherapy, routine follow up study.

Findings:

Right breast

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast

- Scattered fibroglandular densities. (ACR b)
- Thickened skin and coarsen trabeculae.
- UOQ architectural distortion with no speculations, masses, asymmetry , or suspicious microcalcifications.
- **Ultrasound** demonstrated UOQ area of parenchymal disruption underneath the operative scar with no solid , cystic lesions or fluid collections.
- Thickened skin reaching 5 mm and coarsen trabeculae.
- Normal nipple/areola complex.
- Clear left axilla. No pathological infra or supra-clavicular lymph nodes

Conclusion:-

Right breast (BIRADS 1): Normal study.

Left breast (BIRADS 2): Post-therapeutic sequelae.

Recommendation:-

For annual follow up.

Best Regards,
DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations

BIRADS 0: Non-informative study.

BIRADS 1: Normal study, routine screening

BIRADS 2: Benign findings, routine screening

BIRADS 3: Probably benign, for close follow up.

BIRADS 4: Probably malignant, for biopsy.

A: mild suspicion.

B: moderate suspicion.

C: high suspicion.

BIRADS 5: Malignant, for biopsy.

BIRADS 6: Pathologically proven malignant

Suspected Cancer Risk

0 %

0 %

2 %

50 %

98-100%

Signature Dr. Amal Hegazy
PM 1:23 2025/10/09

Patient Name: Namh Ismail Ahmed Ismail	Date : 24/10/2024
Exam : MAMMOGRAPHY BOTH	Age : 50 Year(s)
Medical ID: 4480/2021	Pat. ID: 17618

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION

ULTRASOUND

Indication:

Patient had It CBS

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- No right breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Left breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed left breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no right breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left axillary, infra and supra clavicular regions.

Opinion:

- Left breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the right breast (BIRADS 1)

Yours truly

Signature Dr. Asmaa Zenhom✓

24/10/2024 1:21 PM



Patient Name: Namh Ismail Ahmed Ismail	Date : 24/10/2024
Exam : MAMMOGRAPHY BOTH	Age : 50 Year(s)
Medical ID: 4480/2021	Pat. ID: 17618

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION

ULTRASOUND

Indication:

Patient had It CBS

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- No right breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Left breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed left breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no right breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left axillary, infra and supra clavicular regions.

Opinion:

- Left breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the right breast (BIRADS 1)

Yours truly

Signature Dr. Asmaa Zenhom

24/10/2024 1:21 PM

Patient Name: Namh Ismail Ahmed Ismail	Date : 09/04/2023
Exam : ultrasound BREAST (BOTH SIDE)	Age : 49 Year(s)
Medical ID: 4480/2021	Pat. ID: 17618

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION

ULTRASOUND

Indication:

Patient had CBS

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- No right breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Left breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed left breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no right breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left infra and supra clavicular regions.

Opinion:

- Left breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the right breast (BIRADS 1)

Yours truly

Signature Dr. Aya Deabes

09/04/2023 1:04 PM

مركز اوراق دار السلام
موسم الحضانة

Patient name: Nema IsmaeilHamed

Date: 24/10/2021

Combined bilateral single view mammography and complementary ultrasound

Indications:

Left breast pathologically proven IDC G II.

Technique

- Bilateral MLO (to assess calcifications) and revision of mammography done outside.
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- Scattered fibroglandular densities (ACR B)
- Left breast LOQ irregular spiculated mass with another posteriorly located one, with no related calcifications.
 - Another UOQ area of focal asymmetry is also appreciated.
- Complementary ultrasound shows left breast irregular spiculated hypoechoic mass, seen at 4 o'clock, 5cm from the nipple, 1cm from the skin, measured 1.4x2cm, (index or main mass)
 - Another similar yet much smaller mass is seen more peripheral to the main mass, measured 0.4x0.7cm, being 1.6cm from the nipple.
 - left breast 2 o'clock area of altered parenchyma seen at 2 o'clock, 4cm from the nipple, measured 0.7x0.9cm, consistent to the mammographic asymmetry.
- no right breast solid or cystic lesions
- Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Left pathological LN (being 2 in number), with focal cortical thickness, reactive 3.5mm
- Nonspecific right axillary LN.

Opinion:

- Left breast LOQ pathologically proven multifocal malignant masses (BIRADS 6), along with 2 o'clock altered parenchyma mounting to mass formation,
- Normal right breast study (BIRADS 1)

Yours truly,

Dr. Engy Abdallah.



Patient Name : الأستاذة / نعمة اسماعيل احمد اسماعيل
Patient No. : 27402282102342
Path No. : 2833/21
Sex : Female
Age : 47 Y
Referreing Doc :
Request No. : 137318
Request Date : 18/11/2021
Verification Date : 23/11/2021
Financial Category : نفقة الدولة - اورام دالر السلام
Department : Out Patient

Pathology Report

Clinical Data:

Left breast cancer. Didn't receive chemotherapy.

Nature of the specimen:

Left WLE & axillary clearance.

Gross Examination:

Two specimens were received:

1- Left wire guided WLE specimen measured 9x5.5x4.5cm, partially covered by skin measured 6.5x3.5cm. Cut section revealed a firm greyish white mass measured 2.5x1.7x0.4cm, with least margin is 1cm (medial margin). Also there were very small multiple satellite subcutaneous nodules (5 nodules).

2- Axillary clearance specimens: A fibrofatty tissue piece measured 6.5x4x1cm, containing 17 lymph nodes.

Microscopic examination:

- Histological type: Remnants of invasive duct carcinoma grade II, including all satellite nodules (positive for invasion), and DCIS components of about 10%.
- Positive perineural invasion
- Angioinvasion: No
- Lymph node involvement: All lymph nodes are free (0/17).
- Resection Margins: All margins are free

Diagnosis:

Left WLE & axillary clearance:

Remnants of invasive duct carcinoma grade II, with DCIS components of about 10%.

Positive invasion in satellite nodules

Positive perineural invasion

All lymph nodes are free (0/17).

All margins are free

Stage: pT2, with multiple satellite nodules

Patient Name: Namh Ismail Ahmed Ismail	Date : 26/10/2022
Exam : MAMMOGRAPHY BOTH	Age : 48 Year(s)
Medical ID: 4480/2021	Pat. ID: 17618

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION

ULTRASOUND

Indication:

Patient had CBS in left breast.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B).
- No right breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Left breast LOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed left breast LOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no right breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left infra and supra clavicular regions.

Opinion:

- Left breast LOQ focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the right breast (BIRADS 1)

Yours truly

Signature Dr. Nehal Hamdy

26/10/2022 12:04 PM